



Nursing Practice Guideline

Intramuscular and Subcutaneous Injection

Reference #: NPG50

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation-Wide	Nursing

Introduction

This guideline outlines the procedures and responsibilities of individuals and the health service organisation in relation to Intramuscular and Subcutaneous Injection. It ensures the delivery of a uniform, integrated multidisciplinary approach with management that is consistent with best practice.

Risk Statement

Non-compliance with this guideline will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☐
Breach SCGH Mission & Values	☒	Staff Safety	☒
Other:	☐		

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These guidelines are intended to guide clinical decision making for clinical staff at North Metropolitan Health Service. They are not strict protocols and do not replace clinical judgement. These guidelines should never be relied on as a substitute for proper assessment of the particular circumstances of each case and the needs of each patient. If staff decide, in exercising their clinical judgment, not to follow the guideline, it is recommended that the reasons are clearly documented in the patient's health care record.

Principles

Observe the five moments for hand hygiene and aseptic non-touch technique. Follow the risk assessment outlined SCGOPHCG Infection Prevention and Management Manual - Aseptic Technique Policy. Appropriate Personal Protective Equipment (PPE) must be worn to prevent contamination and mucosal or conjunctival splash injuries.

Patient identification must be checked when matching the patient's identity to care, therapy and services.

1.0 Intramuscular and Subcutaneous Injection

1.1 Legislative Requirements

It is a legal requirement that all disciplines that prescribe or administer medications comply with the Medicines and Poisons Act 2014, Medicines and Poisons Regulations 2016, Pharmacy Act 2010 and Hospital Guidelines. It is a legislative requirement that a valid prescription must exist before Schedule 4 and Schedule 8 medications are administered.⁽¹⁾

A valid prescription must include:

- Doctor's or Nurse Practitioner's signature PLUS printed surname or stamp of a surname.
- A prescriber number is necessary to prescribe medications (Schedule 4 and above).
- The name and address of the patient.
- The date when it was written.
- The name, form, strength and quantity of the medication.
- Directions for use, including route, dose and frequency.

An endorsed Nurse Practitioner who is engaged in a designated area has the legislative authority to prescribe medications within approved clinical protocols.⁽¹⁾

Only valid prescriptions may be utilised by nursing staff to administer medication except for verbal orders and nurse initiated medications, as detailed within the approved register.

All personnel administering medications using these devices must adhere to safe injection principles.

1.2 Medical Requirements

A valid prescription is required by nursing staff before administering medication, except for verbal orders. All prescriptions are to be written on an SCGH approved medication chart or prescription form, prescriptions must be legible and address all legal requirements.

2.0 Intramuscular (IM) Injection

2.1 Safety Information

Intramuscular and Subcutaneous injections (excluding Schedule 8) may be administered by one (1) registered or enrolled nurse, once checked by a 2nd qualified clinician (i.e. Pharmacist, Pharmacy Intern, Medical Officer, RN or EN).

All injectable medicines drawn up in a syringe that will leave the hands of the person preparing the intramuscular injection should be labelled **immediately** with a pink coloured 'miscellaneous' medication label.⁽²⁾ Labelling is not required when the preparation and bolus administration of a single medicine is one continuous process, and where the syringe **does not** leave the hands of the person who prepared it and that same person administers the medicine.

While preparing an intramuscular injection, inspect the packaging of the medication being prepared for damage; check the expiry date and the medications sterility indicators.

The nurse administering the intramuscular injection is responsible for checking whether the recipient patient has known allergies before administration.

Contraindications for intramuscular injection:⁽³⁻⁵⁾

- Coagulopathy,
- Peripheral vascular disease, oedema and, shock as these impair peripheral absorption,
- After thrombolytic therapy or during acute myocardial infarction.

Potential Complications of intramuscular injection:⁽³⁻⁵⁾

- Pain,
- Fibrosis and contracture of muscles,
- Nerve injuries / palsy / neuropathy,
- Arterial puncture / haematoma formation,
- Inadvertent subcutaneous injection,
- Local irritation / infection,
- Abscess / necrosis,
- Periostitis.

The ventrogluteal site is the site of preference for deep IM injections³⁻⁷

Use of the dorsogluteal site for IMI injection is not advised, as it is associated with an increased risk of complications including damage to the sciatic nerve and the superior gluteal arteries.⁽⁶⁻⁹⁾ Muscle mass may have atrophied in older or non-ambulant patients.^(4, 9) Muscle at this site has a lower drug absorption rate, and this can result in a build-up of medication in the tissues, increasing the risk of overdose.⁽⁹⁾ There is also a risk that the medication will not reach the muscle and due to the amount of subcutaneous tissue in this area be administered subcutaneously.^(4, 9)

2.2 Nursing Management

Intramuscular injections are intended to deposit medication deep into the muscle layer; vascularity of the muscle aids the rapid systemic action and absorption of relatively large doses of drugs when administered at appropriate sites.^(3, 9, 10) Potential sites to administer intramuscular injections include the deltoid, dorsogluteal, rectus femoris, vastus lateralis and ventrogluteal area. Choice of the site chosen will be influenced by patient preference, physical condition, medication volume and type; however the ventrogluteal site is preferred.^(3, 9, 11)

Syringe and Needle Selection:

Refer to individual product licences and product information when considering the choice of site and the length and gauge of the needle.^(3, 12)

The size of the syringe and the gauge and length of the needle are determined by several factors, including the volume and viscosity of the medication, the injection site chosen, the patient's body mass and amount of adipose tissue. The patient's BMI, body shape and, gender should be taken into account to determine the most appropriate needle length to reach the muscle layer.^(3, 10-13) Females have thicker subcutaneous fat than males at both the ventrogluteal and dorsogluteal sites⁽¹¹⁻¹³⁾.

Potential variables for intramuscular injection

- 3 or 5ml syringe depending on the volume to be injected.
- Biological agents and medications in aqueous solutions 20G to 25G needle.
- Oil based solutions 18G to 25G needle.
- Adult average weight / adipose tissue 1 inch (25mm) to 1 ½ inch (38mm).
- Obese patients 1 ½ inch (38mm) needle or longer.
- Thin, emaciated patients 5/8 inch (16mm) to 1 inch (25mm) needle.

The needle should be changed after drawing up the medication to ensure the needle is sharp and free from medication residue or particles, unless using a prefilled syringe. A safety needle should be used to minimise the risk of needle stick injury.⁽¹⁰⁾

Skin disinfection is recommended using an 70% isopropyl alcohol skin cleansing swab. Cleanse the skin for 30 seconds and allow to dry for 30 seconds.⁽¹⁵⁾

Aspiration for blood is generally unnecessary and should be avoided as it can cause increased pain.^(14, 15) Particular medications may come with instructions from the manufacturer to aspirate, and this should then be followed.⁽¹⁴⁻¹⁶⁾

The Z track method is the preferred method as it seals the needle track preventing leakage of medications into the tissues, minimising irritation.^(10, 12, 17)

If multiple intramuscular injections are required, rotate the sites.⁽¹²⁾

Position the patient in a comfortable position to relax the muscle to be injected.⁽¹²⁾

Intramuscular Injection Sites

Ventrogluteal Site:

The preferred site for IM injection is the ventrogluteal site where there is thick muscle that is relatively free of major nerves and blood vessels.^(6, 7, 10, 13) The gluteus medius targeted at the ventrogluteal site is capable of absorbing up to 3-4mls of fluid in one injection.⁽¹²⁾ Where larger volumes are required to be injected, consideration should be given to injecting volumes of between 3-4mls in multiple divided doses.⁽¹²⁾

The ventrogluteal site can be used to administer a broad range of drugs including antibiotics, drugs suspended in oils, opioids and, sedatives.^(3, 12) This site can be accessed with the patient lying supine, prone or on their side.^(4, 18)

The V method: (figures 2 & 3)^(4, 10, 13, 15, 19) is reliable for patients with a BMI < 30 ⁽⁷⁾

- Locate ventrogluteal site by placing the palm of hand against the greater trochanter of the femur and the index finger on the anterior superior iliac spine of the pelvis (right hand to left hip, left hand to right hip).
- Abduct the middle finger posteriorly along the iliac crest.
- Inject between the “V” of the two fingers (figure 2). Up to 4mls can be injected.

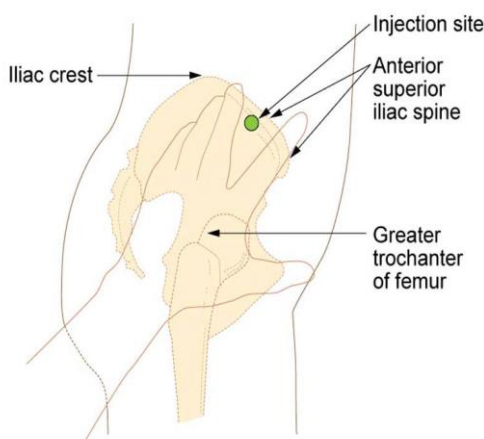


Figure 2: Ventrogluteal Injection

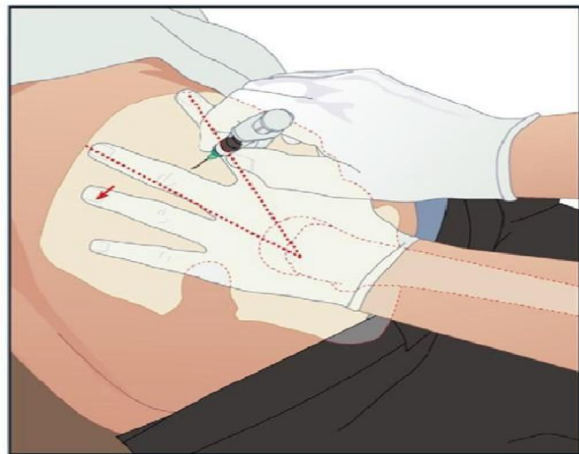


Figure 3: Ventrogluteal injection

Deltoid: Intramuscular injections of 1ml are recommended; however, up to 2ml can be administered (figures 4 & 5).^(12, 19)

The most accessible site but small area limits the number and volume of injections that can be administered.^(9, 19) The deltoid site may be used as the site of choice for vaccinations.⁽¹⁵⁾

- Locate the deltoid muscle - palpate the lower edge of the acromion process which forms the base of the triangle in line with the midpoint of the lateral aspect of the upper arm
- The injection site is in the centre of the triangle, about 2.5–5cm below the acromion process. If the injection is too proximal in the arm, there is a risk of injection into the joint or the tendons of the shoulder, and if too distal in the arm there is a risk of injection into the radial nerve.⁽⁹⁾

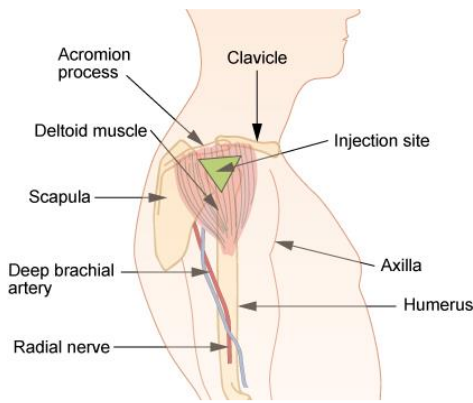


Figure 4: Deltoid injection

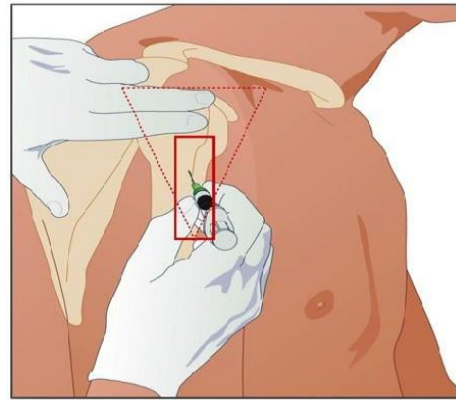


Figure 5: Deltoid injection

Vastus Lateralis and Rectus Femoris (figure 6).⁽¹⁹⁾

Vastus lateralis: Up to 5mls can be injected

No major blood vessels or significant nerve structures are associated with the site. Used for deep IM and Z-track injections.

- Located on the anterior lateral aspect of the thigh and extends from a handbreadth above the knee to a handbreadth below the greater trochanter of the femur,
- The middle third of the muscle is the best site for injection.^(9, 15)

Rectus femoris: Up to 5mL can be injected.⁽¹⁵⁾ The anterior quadriceps muscle is used primarily for antiemetic, opioids, sedatives, injections in oil, deep intramuscular injections. There is a higher chance of intravascular/ intraneural injection when using this site and the vastus lateralis should be used where possible.

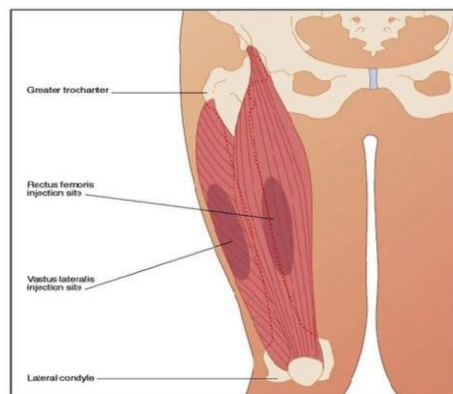


Figure 6: Vastus lateralis and rectus femoris

Methods for Intramuscular Injection

Z Track Method (see procedural information below)

This method prevents backtracking of medications into tissues, decreases pain and injection site lesions, particularly when administering cytotoxic, iron or oil IM injections.^(8, 12)

2.3 Procedural Information

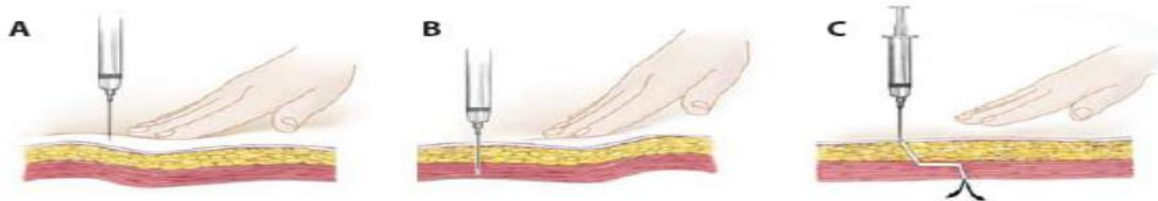
Requirements

- WA hospital medication chart (MR410/805)
- Prescribed medication for administration
- Disposable kidney dish
- Non-sterile gloves if indicated (gloves not required if healthcare workers and patients' skin is intact)
- Sterile 70% alcohol swab
- 3ml or 5 ml syringe
- Single-use sterile drawing up needle
- 18-25 gauge self-sheathing needle for administration (of sufficient length to penetrate the muscle)
- Gauze square
- Small plaster

Procedure

1. Perform hand hygiene.
2. Gather equipment for the procedure in a disposable kidney dish.
3. Perform hand hygiene.
4. Prepare medication, including labelling if required.
5. Explain the procedure to the patient and gain verbal consent. ⁽¹¹⁾
6. Perform hand hygiene.
7. At the bedside confirm the patient identity using 3 points of identification, and check / confirm the unique medical record number (UMRN) on the patient identification band with medication chart.
8. Prior to administration check for allergies.
9. Perform hand hygiene.
10. Select site based on body mass, the purpose of injection and volume of medication to be injected.
11. Ensure the site is not oedematous, inflamed or scarred; avoid moles, birthmarks, scar tissue or other lesions.
12. Position the patient in a comfortable position to relax the muscle to be injected.
13. Perform hand hygiene and don non-sterile gloves (if indicated).
14. Identify the appropriate landmarks for the chosen site.
15. Clean site for 30 seconds with 70% alcohol swab and allow to air dry for 30 seconds.
16. Remove the cap from the needle.

Z-Track Method - This is the preferred technique for all IM injections (Figure 7).^(8, 10, 12, 20)



17. Pull skin at the injection site taught and to one side (~1–2cm) with the thumb or side of the non-dominant hand and maintain this firm traction of the skin throughout the procedure.
18. Hold the needle like a dart and quickly plunge the needle into the muscle at an angle of 90°; there is no need to aspirate.
19. Slowly administer medication at approximately one ml every 10 seconds to allow the muscle fibres to expand to accommodate the drug.

20. Wait a further 10 seconds before removing the needle and once it has been removed, release the traction (Z-tracking) on the skin.
21. Remove the needle and engage the safety needle cap.
22. Apply pressure for 20 seconds with a gauze swab, DO NOT massage site
23. If any bleeding occurs, apply small plaster to site.
24. Dispose of sharps and waste appropriately.
25. Remove gloves if worn and perform hand hygiene.
26. Document administration on the WA hospital medication chart.
27. Evaluate for any adverse response to medication or adverse reaction at the injection site.

3.0 Subcutaneous (SC) Injection

3.1 Safety Information

SC injections (excluding Schedule 8) may be administered by one (1) registered or enrolled nurse, once checked by a 2nd qualified clinician (i.e. Pharmacist, Pharmacy Intern, Medical Officer, RN or EN).

All injectable medicines drawn up in a syringe that will leave the hands of the person preparing the subcutaneous injection should be labelled **immediately** with a brown coloured 'miscellaneous' medication label.². Labelling is not required when the preparation and bolus administration of a single medicine is one continuous process, and where the syringe **does not** leave the hands of the person who prepared it and that same person administers the medicine.

While preparing an SC injection, inspect the packaging of the medication being prepared for damage; check the expiry date and the medication sterility indicators.

The nurse administering the SC injection is responsible for checking whether the recipient patient has known allergies before the delivery of the injection.

Insulin and heparin are considered high risk medications because they cause significant harm when used in error. Two nurses are required to independently check and confirm the dosage of and prescription prior to administration of subcutaneous Insulin or Heparin.

If administering a medication recognised as a hazardous drug, consider personal protective equipment requirements.

Potential Complications of subcutaneous injection:

- Concentrated and irritating solution can cause sterile abscesses,
- Repeated injections at same site can cause lipodystrophy ²¹.

3.2 Nursing Management

Medications administered by the SC route are deposited beneath the epidermis into the fat and connective tissue underlying the dermis. This allows for a slow and steady absorption rate (Fig 8); small volumes (up to 2mls) of non-irritant, water-soluble medications can be administered. ^{22,23}

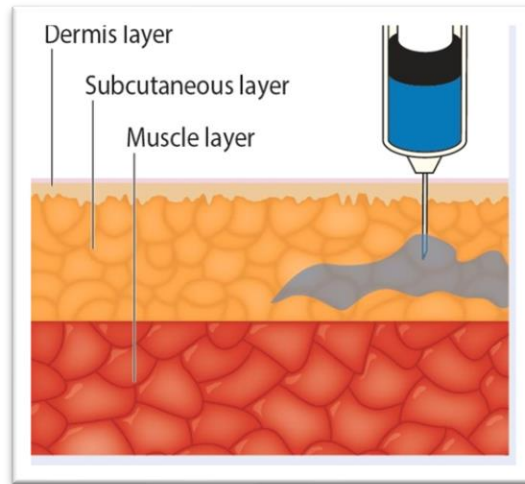


Figure 8: Tissue structure and subcutaneous injection. ²¹

Aspiration for blood is not recommended, as administration into the bloodstream is rare.^{21,22}

Site Selection

Recommended sites for SC injection include the umbilical region of the abdomen, lateral aspects upper arm and thigh, upper hips, and upper back.²³ (Figure 9)

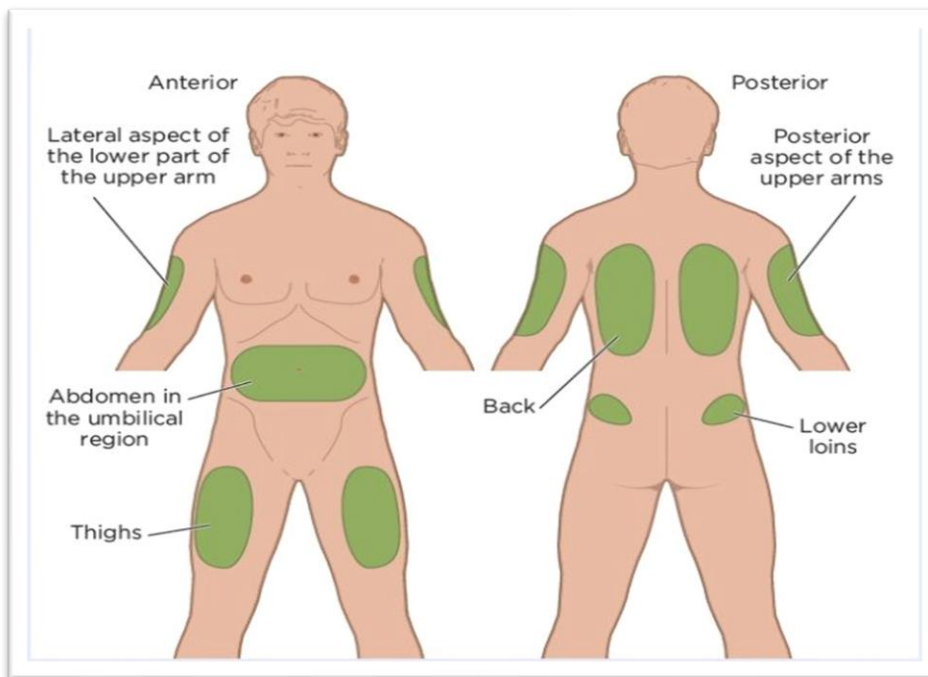


Figure 9: Subcutaneous injection sites ²¹

Injection sites should be:

- Clean,
- Free from infection, skin lesions, scars, birthmarks, bony prominences, and large underlying muscles, blood vessels and nerves.²²

The most commonly used site is the abdomen as it has thicker SC tissue and faster absorption,²³ avoid the umbilicus by a 5 cm radius,²⁴ for repeated injections rotate the site.^{21,22}

Syringe and Needle Selection

A 25g needle is commonly used for SC injection – consider the viscosity of the liquid being injected when selecting needle size and refer to manufacturer's instructions.²²

The needle should be changed after drawing up the medication to ensure the needle is sharp and free from medication residue or particles, unless using a prefilled syringe. A safety needle should be used to minimise the risk of needle stick injury.

Skin Preparation

In the hospital-setting, if the skin is visibly soiled it should be cleaned prior to injection using an alcohol impregnated swab (70% alcohol).²⁶

Method and Angle of Injection

Injection at 90 degrees is recommended to reduce the risk of inadvertent intramuscular injection. Patients who are cachectic and have minimal amounts of SC tissue may require administration at a 45-degree angle.

Use a lifted skinfold technique by pinching or bunching the skin to lift the subcutaneous layer away from the underlying muscle (figure 10).²⁵

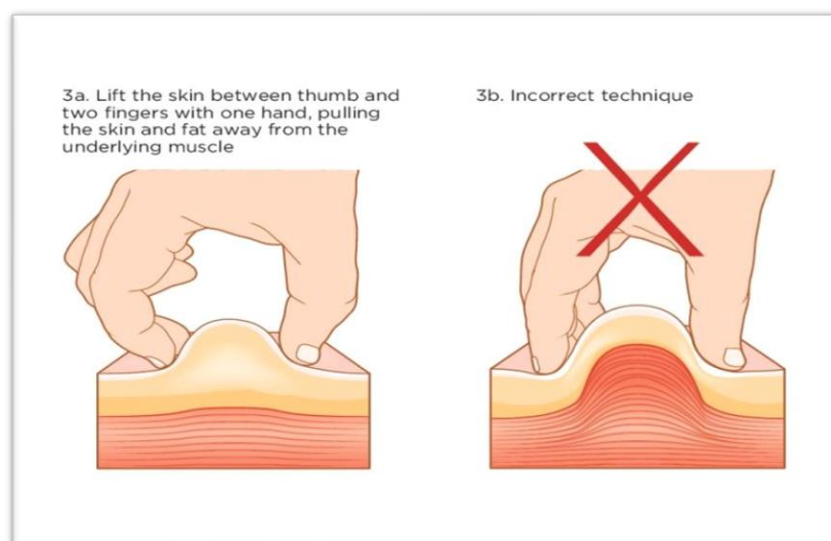


Figure 10 – Lifted Skinfold Technique²¹

3.3 Procedural Information

Requirements

- WA hospital medication chart
- Prescribed medication for administration
- Disposable kidney dish
- 1-3ml syringe
- Single-use sterile drawing up needle
- 25g ½ to 5/8-inch self-sheathing needle for administration
- (Or prefilled syringe and needle)

- Sterile 70% alcohol swab (if skin visibly soiled)
- Gauze square
- Small plaster
- Optional; Non-sterile gloves if indicated (gloves not required if healthcare workers and patients skin is intact) ²⁶
- Medication label (if medication is leaving hands).

Procedure

1. Perform hand hygiene.
2. Gather equipment for the procedure in a disposable kidney dish.
3. Perform hand hygiene.
4. Prepare medication, including labelling if required.
5. Explain the procedure to the patient and gain verbal consent.
6. Perform hand hygiene.
7. At the bedside confirm the patient identity using 3 points of identification, and check/ confirm the unique medical record number (UMRN) on the patient identification band with medication chart.
8. Prior to administration check for allergies.
9. Perform hand hygiene.
10. Select site, rotate site if repeated injections.
11. Position the patient in a comfortable position.
12. Perform hand hygiene and don non-sterile gloves (optional).
13. If visibly soiled, clean skin with sterile 70% alcohol swab; for 30 seconds and allow to dry for 30 seconds. ²¹
14. Remove the cap from the needle.
15. Hold the syringe and needle in your dominant hand and pinch the skin together using the non-dominant hand to lift the tissue away from underlying muscle (Fig 10) ²²
16. Insert the needle at the required angle (usually 90 degree) using a dart-like action. Aspiration to check whether the needle is in a blood vessel is not necessary
17. Slowly administer medication at approximately one ml every 10 seconds to allow the tissues to expand to accommodate the drug. ^{22,27}
18. Wait a further 10 seconds before withdrawing the needle to prevent backtracking of the medication ²⁸ then withdraw the needle and engage the safety needle cap. Do not rub the area ²³ Release the skin fold.
19. If any bleeding occurs, apply pressure using gauze square for 20 seconds and apply small plaster to site.
20. Dispose of sharps and waste appropriately.
21. Remove gloves if worn and perform hand hygiene.
22. Document administration on the WA hospital medication chart.
23. Evaluate for any adverse response to medication or adverse reaction at the injection site.

Related Documents

Legislation:

- Health Practitioner Regulation National Law (WA) Act 2010
- Medicines and Poisons Act 2014, Medicines and Poisons Regulations 2016, Pharmacy Act 2010
- Work Health and Safety Act 2020
- Nursing and Midwifery Board of Australia (NMBA) Code of Professional Conduct for Nurses March 2018

Policy:

- WA Health Mandatory Policy Consent to Treatment Policy
- NMHS Consent to Treatment Policy
- NMHS Clinical Documentation Policy
- SCGOPHCG Patient Identification Policy
- SCGH Procedure Matching Policy
- OPH Procedure Matching Policy
- SCGOPHCG Infection Prevention and Management Manual - Aseptic Technique Policy
- SCGOPHCG Infection Prevention and Management Manual – Safe Injection Principles Policy

Guidelines:

- SCGOPHCG Nursing Practice Guideline Diabetes Patient Management,
- SCGOPHCG Nursing Practice Guideline Antineoplastic / Hazardous Agent Management (in the cancer and non-cancer setting)
- SCGOPHCG Nursing Practice Guideline Medication Management.

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